Fax cover sheet

Silver Mesa General Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Gary Nwosu, RN CM
Phone: 555-0161
Date: 10/06/2026 10:20
Pages (including cover): 13
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

76F admitted with E. coli UTI and delirium, delirium resolved. Now on oral antibiotics. CalAIM authorization submitted to plan 10/05, pending; will forward when received.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Silver Mesa General Hospital (fictional)
Dept/Floor: 3 North Medicine
SW/CM/RN: Gary Nwosu, RN CM
Phone/Pager #: 555-0161
Email: gnwosu@silvermesa.example
Nursing Station #: 555-0162
Authorized by: Dr. L. Moreau
Phone #:

Patient Information

Patient Name: Dolores Doe MRN: SM-2290415 Date of Birth (dd/mm/yyy): 11/30/1949
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: Hispanic/Latino
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English, Spanish
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
UTI with acute metabolic encephalopathy (delirium)
Admit Date: 10/02/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/02/2026 Results: CXR neg

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes*	X No	Active Infection	X Yes*	No
*Explain			*Explain	UTI - PO abx	
Self Administer All Meds	X Yes	No*	Ambulatory	X Yes	No
Continent of Bowel & Bladder	X Yes	No	Assistive Device	Yes*	X No
Colostomy/Ileostomy	Yes	X No	*Explain		
Foley Catheter	Yes	X No	Hospice	Yes*	X No
Diabetes	Yes	X No	*Explain		
Insulin	Yes	X No	Car	Yes	X No
Diabetes related Oral Meds	Yes	X No	Service Animal	Yes	X No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? X No Yes
☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain None

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

Patient face sheet

Silver Mesa General Hospital (fictional) - PATIENT FACE SHEET

Patient: Dolores Doe
MRN: SM-2290415
Account #: 99290415
DOB: 11/30/1949
Sex: Female
Preferred Language: Spanish
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/02/2026 13:15 (ED arrival 10/02/2026 08:50)
Admit Type: Inpatient
Unit/Bed: 3N-318
Attending: Louise Moreau, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

Alerts

Fall risk: YES
Delirium precautions: YES (added 10/02/2026)

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL

Date: 10/02/2026

CC

Confusion, fever

HPI

76 y/o F, unhoused (staying at a church parking lot shelter program), brought in by shelter staff for 2 days of new confusion, fever and urinary frequency. Shelter staff report she is usually "sharp" and manages her own pill box. In ED T 101.4F, disoriented to place and time, UA grossly positive.

PMH

- Hypertension
- Hyperlipidemia
- Osteoarthritis of knees
- No prior diagnosis of dementia

Meds PTA (per shelter pill box)

- Amlodipine 5 mg daily
- Atorvastatin 20 mg daily

Allergies

Sulfa (rash)

Social

Never smoker. No alcohol or drugs. No family contact listed.

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Exam

T 101.4F HR 104 BP 142/76 RR 18 SpO2 97% RA
Drowsy, oriented to person only, inattentive (CAM positive). Suprapubic tenderness. No focal neuro deficit.

Data

WBC 14.1 H, Cr 1.3 (baseline unknown), Na 133, Glu 112
UA: + leukocyte esterase, + nitrite, >50 WBC
CT head: no acute abnormality, mild age-appropriate atrophy

Assessment/Plan

1. Acute cystitis/pyelonephritis with sepsis physiology - ceftriaxone 1 g IV daily, urine culture pending, IV fluids
2. Acute metabolic encephalopathy (delirium) secondary to infection - delirium precautions, avoid sedatives, reorient
3. HTN - hold amlodipine while on fluids
4. Dispo - PT/OT when appropriate

Signed: Louise Moreau, MD 10/02/2026 15:40

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing note 10/04

NURSING SHIFT NOTE

Date: 10/04/2026 07:00 (night shift 10/03-10/04)
Pt pulled at PIV x1 around 01:00, redirected easily, calm, no aggression. Did not use call light; incontinent of urine x1 at 02:30, brief changed by CNA.

NURSING SHIFT NOTE

Date: 10/04/2026 19:00 (day shift)
Pt AOx3 (person, place, situation); unable to state month or year.
Afebrile. Ambulated to bathroom with RN standby. Ate 75% of meals.
Signed: H. Castellanos, RN

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing note 10/05

NURSING SHIFT NOTE

Date: 10/05/2026 19:00 (day shift)

Pt AOx4 this shift. Pleasant, conversant.

Pt needed reminders at 09:00 and 13:00 to take scheduled meds; meds administered by RN. Ambulated in hallway with walker. Voiding in bathroom.

Signed: M. Ferreira, RN

NURSING SHIFT NOTE

Date: 10/06/2026 06:00 (night shift 10/05-10/06)

Pt woke at 03:00 and asked where she was; reoriented, went back to sleep. No agitation.

Signed: K. Obi, RN

-- FICTIONAL DEMO RECORD - not a real patient --

Physical therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 10/05/2026 11:00
PLOF (per shelter staff): independent ambulation, no device
Bed mobility: modified independent with rail
Transfers: sit<>stand modified independent with FWW
Gait: 180 ft with front-wheeled walker (FWW), modified independent, 1 standing rest break
Balance: fair, mild unsteadiness without device
Recommendation: FWW for community distances. No further acute PT needs. Discharge with FWW.
Signed: S. Haddad, PT, DPT

-- FICTIONAL DEMO RECORD - not a real patient --

Occupational therapy evaluation

OCCUPATIONAL THERAPY EVALUATION

Date: 10/05/2026 14:30
Grooming: independent seated at sink
UB dressing: independent
LB dressing: independent with extra time
Toilet transfer: modified independent with grab bar
Toileting hygiene: not observed this session
Shower: not assessed; pt declined, bed bath by CNA this AM
Medication management: not assessed
Cognition: alert, follows 2-step commands, slowed processing; formal cognitive screen deferred until delirium fully clears
Recommendation: re-assess shower and medication management before discharge.
Signed: P. Lindgren, OTR/L

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5

Date: 10/06/2026 08:15

S: Feels "much better." Eating well. Denies dysuria.

O: T 98.1F HR 80 BP 134/78 RR 16 SpO2 97% RA. Alert, conversant, attention improved. Intermittent nighttime disorientation per nursing.

Urine cx 10/02: E. coli, pan-sensitive except ampicillin.

A/P:

1. E. coli UTI - afebrile since 10/03; ceftriaxone 10/02-10/04, now cefpodoxime 200 mg PO BID through 10/09
2. Delirium - resolving, not fully resolved; no psychiatric history; no SI. Outpatient cognitive screening once recovered.
3. HTN - amlodipine resumed 10/05

Medically stable for discharge. CM pursuing recuperative care.

Signed: Louise Moreau, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Case management note

CASE MANAGEMENT NOTE

Date: 10/05/2026 16:00

Pt unhoused, staying at church shelter program, cannot return there while recovering (no daytime access).

Pt agreeable to recuperative care. No family involved. No conservatorship on file. No history of FSP per pt.

Shelter staff state pt managed her own pill box before admission.

Signed: Gary Nwosu, RN CM

-- FICTIONAL DEMO RECORD - not a real patient --

Vitals and labs

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/05 08:00	98.4	84	138/80	16	97%	RA
10/05 16:00	98.6	82	136/78	16	96%	RA
10/05 23:00	98.2	78	130/74	16	97%	RA
10/06 04:00	98.0	76	128/74	16	97%	RA
10/06 08:15	98.1	80	134/78	16	97%	RA

LABS

10/02 | WBC 14.1 H | Hgb 12.2 | Plt 240 | Cr 1.3 H | Na 133 L | Glu 112
10/05 | WBC 7.9 | Cr 0.9 | Na 137 | K 4.1
10/02 Urine cx: E. coli >100,000 CFU, pan-sensitive except ampicillin
10/02 Blood cx x2: no growth at 72 hours
10/02 MRSA nares PCR: Negative
10/06 05:30 SARS-CoV-2 rapid antigen: Negative
10/02 CXR: no acute disease

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list

DISCHARGE MEDICATION LIST (DRAFT)

Patient: Dolores Doe MRN: SM-2290415

- Cefpodoxime 200 mg PO twice daily through 10/09/2026
- Amlodipine 5 mg PO daily
- Atorvastatin 20 mg PO daily
- Acetaminophen 650 mg PO every 6 hours PRN pain

DME: front-wheeled walker (ordered)

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.